

Tuberculosis Exposure Control

Policy Number	POL-IC-026	Owner / Department	Infection Prevention & Control / Employee Health
Effective Date	2023-07-15	Version	3.0
Last Revised	2026-03-01	Next Review	2028-03-01
Approved By	Infection Prevention & Control Committee	Applies To	All staff, providers, and areas where patients with suspected or confirmed tuberculosis may be encountered

1. Purpose

To prevent transmission of *Mycobacterium tuberculosis* to patients and staff through early identification, airborne isolation, respiratory protection, and employee testing, consistent with CDC guidance and the OSHA respiratory protection standard.

2. Definitions

- **Active (infectious) TB** — clinically active disease, typically pulmonary or laryngeal, capable of airborne transmission.
- **Latent TB infection (LTBI)** — infection without active disease; not infectious.
- **AIIR** — airborne infection isolation room at negative pressure with ≥ 12 air changes per hour.
- **IGRA / TST** — interferon-gamma release assay or tuberculin skin test used for baseline and post-exposure screening.

3. Policy and Procedure

1. Triage: identify patients with signs/symptoms of TB (cough > 3 weeks, fever, night sweats, weight loss) and mask them immediately.
2. Place any patient with suspected or confirmed infectious TB in an AIIR on Airborne Precautions; keep the door closed.
3. All staff entering the room wear a fit-tested N95 or PAPR; fit testing is performed at least annually per OSHA 29 CFR 1910.134.
4. Discontinue airborne isolation only when TB is excluded or the patient meets criteria for non-infectiousness as determined by Infection Prevention and the treating provider.

IMPORTANT — A patient with suspected or confirmed infectious TB must be placed in an AIIR with the door closed before any non-masked contact, and all entrants must wear a fit-tested N95 or PAPR. If no AIIR is available, mask the patient, limit entries, and contact Infection Prevention immediately to arrange transfer.

4. Employee Testing and Exposure

Situation	Action
Baseline (on hire)	Individual TB risk assessment, symptom evaluation, and an IGRA or two-step TST.
Annual	No routine testing of asymptomatic staff absent exposure or ongoing risk; annual TB education for all.
After unprotected exposure	Symptom screen and baseline test if not known positive; repeat IGRA/TST 8–10 weeks after the last exposure.

5. Severity & Reporting

TB events follow POL-RM-013. An unprotected staff or patient exposure to a patient with confirmed infectious TB, or a confirmed healthcare transmission, is escalated as SEV-1 — notify the Nursing Supervisor, Risk Management, Infection Prevention, Employee Health, and the Administrator-on-Call immediately and within 1 hour; Infection Prevention reports confirmed TB to the state health department. An isolation or respiratory-protection breach requiring an exposure investigation is SEV-2 (notify within 4 hours). A breach caught before exposure is a Near Miss, reported within 24 hours. Exposed staff follow-up is coordinated by Employee Health, including repeat testing at 8–10 weeks.

6. Documentation Requirements

- Patient triage, masking, AIIR placement, and isolation start/stop times.
- Staff respirator fit-test records and N95/PAPR use.
- For an exposure: list of exposed persons, baseline and follow-up test results, assigned severity, public-health notification, and all notifications.

7. References

- CDC, Guidelines for Preventing the Transmission of *M. tuberculosis* in Health-Care Settings, MMWR 2005; CDC/NTCA Testing of Health Care Personnel (2019).
- OSHA Respiratory Protection Standard, 29 CFR 1910.134 (fit testing, medical evaluation).
- CMS Conditions of Participation, 42 CFR 482.42.
- Related: POL-RM-013, POL-IC-024 (isolation), POL-EH-001 (occupational exposure).